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The UN Convention on the Rights of Persons with Disabilities: 'Rights, will and preferences' in relation to mental health disabilities

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ABSTRACT

The UN Convention on the Rights of Persons with Disabilities spells out in the most up-to-date and specific manner the rights of persons with disabilities. In doing so the Convention presents serious challenges to many conventional practices in respect of people with disabilities, especially those with mental health disabilities. According to a number of authorities, most notably the UN Committee on the Rights of Persons with Disabilities, 'substitute decision-making' is not consistent with the Convention. 'Respect for the rights, will and preferences' is to be regarded as the touchstone for compliance with the Convention. However 'will and preferences' is nowhere defined. This paper examines the meaning of these terms, and draws attention to instances where a contemporaneous 'preference' may be at variance with a person's 'will'. In such cases, it is argued, that 'substitute decision-making' – or giving effect to the person's 'will' – may be justified.

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The Convention on the Rights of Persons with Disabilities (CRPD) adopted by the United Nations in 2006 (United Nations, 2006) spells out in the most up-to-date and tailored manner the rights of persons with disabilities. In doing so the Convention presents serious challenges to many conventional practices in respect of people with disabilities, especially those with mental health disabilities. One that is far-reaching is involuntary or non-consensual treatment. Whether this can ever be justified within the terms of the Convention is the subject of this paper.

1. The Convention

The overall purpose of the Convention, stated in Article 1, is to

> promote, protect and ensure the full and equal enjoyment of all human rights and fundamental freedoms by all persons with disabilities, and to promote respect for their inherent dignity.

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The elimination of discrimination by ensuring that rights may be enjoyed 'on an equal basis with others' is a fundamental aim. Among the principles are: respect for the inherent dignity, individual autonomy including the freedom to make one's own choices, and independence of persons; full and effective participation and inclusion in society; respect for difference and acceptance of persons with disabilities as part of human diversity and humanity; and equality of opportunity. Their relevance to the standing of persons with mental health disabilities is clear.

The CRPD characterizes 'disability' as follows:

> Persons with disabilities include those who have long-term physical, mental, intellectual or sensory impairments which in interaction with various barriers may hinder their full and effective participation in society on an equal basis with others (Article 1).

This is not an exhaustive definition. However, it is accepted by almost all authorities that persons with a mental disorder which is long-term and which is likely to result in the person being treated within the mental health system are to be included as potentially having a 'disability'. Disabilities associated with 'serious' mental illnesses – for example, a psychosis, bipolar illness, severe depression, or autism are sometimes termed 'psychosocial' disabilities.

The model of disability evident in the characterization quoted above is called a 'social model'. In this interactional model, it is the level of accommodation made by a society that determines the degree to which an impairment becomes a disability.

As of July 2015 Convention had been signed by 159 State parties, ratified by 156, with 92 signatories to the Optional Protocol. By signing the Convention State parties indicate their intention to take steps to be bound by the treaty at a later date. Signing also creates an obligation to refrain from acts that would defeat the object and purpose of the treaty. The next step is 'ratification'. Ratification signals the willingness of the state to undertake the legal rights and obligations contained in the Convention.

The Convention establishes the UN Committee on the Rights of Persons with Disabilities (CRPD Committee), to which signatory State parties are to report periodically about their progress in its implementation. The Committee in turn publishes its observations and recommendations (called 'Concluding Observations') concerning this progress. The Convention requires governments to ensure that representatives of civil society, in particular persons with disabilities, are fully involved in this monitoring. State parties signing an 'Optional Protocol' recognize the competence of the Committee to examine alleged violations of the Convention from individuals when local processes have become exhausted.

Depending on the jurisdiction, the Convention may or may not be automatically incorporated into national or domestic law upon its ratification. In many common law countries (like the United Kingdom) it is incorporated into domestic law only when directly legislated.

2. The CRPD, detention and involuntary treatment

The position of people with mental illness or disabilities will be radically transformed if the rights spelt out in the Convention are to be put into practice. There is, in addition, a significant challenge to justifications for detention and treatment without consent or against the objection of the person.

The CRPD Committee, charged with issuing authoritative interpretations of key articles in the Convention, has done so for Article 12, *General Comment on Article 12: Equal recognition before the law* (United Nations Committee on the Rights of Persons with Disabilities, 2014a).[1] This pivotal Article states that all persons enjoy 'legal capacity' in all aspects of life on an 'equal basis with others'. Article 12(3) also recognises the obligation of states to ensure access to the supports a person may require to exercise 'legal capacity'. The Committee maintains that the right to 'legal capacity' encompasses both the ability to 'hold rights and duties (legal standing) and to exercise those rights and duties (legal agency)'. 'Legal capacity' is taken to mean the legal recognition of a range of acts such as the right to vote, to enter into contracts, to write a will, to marry, and so on.

Contrary to the virtually universal provisions in mental health and capacity law, the Committee states that the existence of an impairment (including a physical, mental, sensory or psychosocial impairment) must never be grounds for denying legal capacity and the imposition of 'substitute decision-making'.[2]

> 36. Respecting the right to legal capacity of persons with disabilities on an equal basis includes respecting the right of persons with disabilities to liberty and security of the person. The denial of the legal capacity of persons with disabilities and their detention in institutions against their will, either without their consent or with the consent of a substitute decision-maker, is an ongoing problem. This practice constitutes arbitrary deprivation of liberty and violates articles 12 and 14 of the Convention.

The Committee insists that the preservation of 'legal capacity' means that we 'must respect the rights, will and preferences of persons with disabilities'. Interventions should never amount to 'substitute decision-making'. The Committee takes the view that all persons retain 'legal capacity' and that with the right level of support people with disabilities will be able to express their 'will and preferences'. The General Comment interpretation makes it clear that 'legal capacity' and 'mental capacity' are to be seen as distinct concepts. The former is a legal concept, the latter a psychological one. The Committee rejects impaired 'mental capacity' as a basis for denial of legal capacity and rejects 'best interests', as it is conceptualized by the Committee, as a basis for substitute decision-making.

The Committee's view on 'substitute decision-making' is further emphasized in its *Guidelines on Article 14 of the Convention on the Rights of Persons with Disabilities: the right to liberty and security of persons with disabilities* (United Nations Committee on the Rights of Persons with Disabilities, 2015).

> III. The absolute prohibition of detention on the basis of impairment
>
> 6. There are still practices in which States parties allow for the deprivation of liberty on the grounds of actual or perceived impairment. In this regard the Committee has established that article 14 does not permit any exceptions whereby persons may be detained on the grounds of their actual or perceived impairment. However, legislation of several States parties, including mental health laws, still provide instances in which persons may be detained on the grounds of their actual or perceived impairment, provided there are other reasons for their detention, including that they are deemed dangerous to themselves or others. This practice is incompatible with article 14; it is discriminatory in nature and amounts to arbitrary deprivation of liberty.

[1] The Committee's interpretations are considered to be 'authoritative' but not 'legally binding'. See Martin et al. (2015) for further details. The General Comment on Article 12 has been criticized by, for example, Dawson (2015) and Freeman, Kolappa, de Almeida, et al. (2015).

[2] The Committee defines 'substitute decision-making' as follows: '(i) capacity is removed from a person, even if this is in respect of a single decision; (ii) a substitute decision-maker can be appointed by someone other than the person concerned, and this can be done against his or her will; and (iii) any decision made by a substitute decision-maker is based on what is believed to be in the objective 'best interests' of the person concerned, as opposed to being based on the person's own will and preferences'. It is still possible under these terms for a person to be appointed to take decisions for another person. Examples might be where a person appoints another person to take a decision based on the appointer's will and preferences, or where a person's will and preferences is unascertainable, a decision should be based on the 'best interpretation' of the person's will and preferences, using whatever information about the person is available.

IV. Involuntary or non-consensual commitment in mental health institutions

10. … The Committee has repeatedly stated[3] that States parties should repeal provisions which allow for involuntary commitment of persons with disabilities in mental health institutions based on actual or perceived impairments.

VI. Protection of persons with disabilities deprived of their liberty from violence, abuse and ill-treatment

12. The Committee has called on States parties to protect the security and personal integrity of persons with disabilities who are deprived of their liberty, including by eliminating the use of forced treatment, seclusion and various methods of restraint in medical facilities, including physical, chemical and mechanic restrains. The Committee has found that these practices are not consistent with the prohibition of torture and other cruel, inhumane or degrading treatment or punishment against persons with disabilities pursuant to article 15 of the Convention.

VII. Deprivation of liberty on the basis of perceived dangerousness of persons with disabilities, alleged need for care or treatment, or any other reasons.

13. … The involuntary detention of persons with disabilities based on risk or dangerousness, alleged need of care or treatment or other reasons tied to impairment or health diagnosis is contrary to the right to liberty, and amounts to arbitrary deprivation of liberty.

The Committee also insists that the 'will and preference' paradigm must replace the 'best interests' paradigm to ensure that persons with disabilities enjoy the right to 'legal capacity' on an equal basis with others.

While the Committee's accounts of 'substitute decision-making' and 'supported decision-making' appear reasonably clear, it has never provided an interpretation of exactly what is meant by 'respect for the rights, will and preferences' of persons with disabilities. The meaning of 'rights' is fairly clear – they are listed in articles of the Convention. What is meant by 'will and preferences', is far from evident. Most of the following discussion will focus on 'will and preferences'.

2.1. A legal meaning of the 'will'?

The 'will' - in the sense discussed here, separate from the common one concerning the disposal of an estate after death - has no clear legal meaning in the vast majority of legal systems.

An exception is in German law. In 1992 adult guardianship (*Vormundschaft für Volljährige*) was replaced by a different form of legal representation for a person seen as seriously vulnerable and in need of protection – *Betreuung* (Brosey, 2015). This form of representation, directed at specific decision areas stipulated by the court, is aimed at maximizing as far as possible the person's autonomy through according precedence to the person's wishes and offering various forms of advice and support. In general the *Betreuer* must comply with the wishes of the person, but as a last resort, the person's expressed wishes can be overridden and a 'substitute decision' made. This is permitted when the person is exposed to substantial danger risking the suffering of serious harm but is unable to recognize the necessity of the protective decision or measure. The approval of the court is necessary for certain decisions.

Two legal terms used in connection with *Betreuung* incorporate the word for 'will' – *freien Willen* ('free will') and *natürlichen Willen* ('natural will'). The former is understood as a competently formed and communicated 'will' – the person understands the nature and consequences of his or her decision and is able to utilise or weigh the information needed for decision-making. The latter refers to a wish (or will) that is consciously and willfully expressed, but that is not capably formulated and communicated. These terms are used in an important July 2016 judgment in the First Senate of the German Federal Constitutional Court (*Bundesverfassungsgericht*).[4] This case turned on the legal justification for the involuntary treatment of a woman with a 'schizoaffective psychosis' and serious physical illness, which would require her placement where she would be deprived of her liberty. The court ruled that the state's obligation to provide protection to a person who has a *Betreuer*, who is vulnerable and faces serious harm, and who is unable to form a 'free will' permits the person's 'natural will', if impaired in its formation by the effects of an illness, to be overridden. The person's 'original free will' – a competent 'will' formed in the past - is to be given precedence. Support may be needed to enable the person to express that 'free will'. The court regarded their judgment as compliant with the CRPD.

We shall return to these concepts of 'will' later.

3. How can the challenges of the Convention be met?

What is clear, on the basis of the interpretations of the Committee noted above, is that any law that allows an interference with the rights of a person that is restricted to persons with a disability is discriminatory.[5] Mental health legislation, virtually everywhere, transgresses this injunction. 'Mental disorder' in one form or another appears on the face of the legislation. No matter how many other criteria are specified - whether the law additionally only permits the person's detention or involuntary treatment in the interests of the person or to protect others - in thus singling out a particular group of people with a disability for special interference, such legislation in the view of the Committee breaches the Convention. Similarly, 'substitute decision-making' based on impaired mental decision-making capacity as is

[3] This is clear from its published 'Concluding Observations' following its examination of reports of progress from States parties in implementing the Convention. In the more than forty published at the time of writing, the Committee has regularly concluded that states must take action to develop laws and policies to replace regimes of 'substitute decision-making' by 'supported decision-making' which respect the person's autonomy, will and preferences. The meaning of 'supported decision-making' –better expressed as 'support for legal capacity' – is not defined by the Committee beyond insisting it be based on respect for the person's will and preferences and be tailored to the person's individual needs. This is largely because it is an evolving practice, whose specific aspects have yet to be defined, but which is generally taken to involve both informal and formal supports aimed at assisting the person to make a decision based on his or her will and preferences. The need for safeguards is recognised (for example against undue influence) (Arstein-Kerslake & Flynn, 2015).

[4] Bundesverfassungsgericht, Beschluss (des ersten Senats) vom 26. Juli 2016 – 1 BvL 8/15. http://www.bundesverfassungsgericht.de/SharedDocs/Entscheidungen/DE/2016/07/ls20160726_1bvl000815.html;jsessionid=52EFD9A94906088EAF0EE8A7E42EBEF6.2_cid370. See also discussion of this case by Adrian Ward: http://www.lexology.com/library/detail.aspx?g=7fdc889d-f50b-4d59-b84a-3bdb4bb75a51.

[5] It can be noted that the Committee's interpretation is at variance with some other UN bodies' statements. In December 2014 the *Human Rights Committee* published a General Comment (No. 35) on Article 9 (Liberty and security of the person) of the *International Covenant on Civil and Political Rights* (United Nations, 2014b). It states '19. The existence of a disability shall not in itself justify a deprivation of liberty but rather any deprivation of liberty must be necessary and proportionate, for the purpose of protecting the individual in question from serious harm or preventing injury to others. It must be applied only as a measure of last resort and for the shortest appropriate period of time, and must be accompanied by adequate procedural and substantive safeguards established by law. The procedures should ensure respect for the views of the individual and ensure that any representative genuinely represents and defends the wishes and interests of the individual. States parties must offer to institutionalized persons programmes of treatment and rehabilitation that serve the purposes that are asserted to justify the detention.' A similar position is taken in *Approach of the Subcommittee on Prevention of Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment regarding the rights of persons institutionalized and treated medically without informed consent* (United Nations, 2016): 'Any deprivation of liberty must be: necessary and proportionate, for the purpose of protecting the person in question from harm or preventing injury to others. It must take into consideration less restrictive alternatives, and must be accompanied by adequate procedural and substantive safeguards established by law'.

commonly found in guardianship legislation is also ruled out on the Committee's account.

4. 'Respect for rights, will and preferences'

Article 12(4) of the Convention states that '… safeguards shall ensure that measures relating to the exercise of legal capacity respect the rights, will and preferences of the person ….' In its interpretation of Article 12, the Committee places huge weight on this formulation. However, it has not offered a definition of what comprises the 'will and preferences' that one must respect.

So, what can those words mean? Much has been written about the 'will' in the philosophy of mind and action. In a recent book, *Action, Knowledge and Will*, Hyman (2015) , traces concepts of the 'will'. He describes the 'incomplete demise' of the 'modern theory of the will' that held sway from Descartes to the 19th century and which came under fierce attack in the 20th century. Locke's notion is an example from the 17th century: 'an act of the Mind, directing its thought to the production of any action, and thereby exerting its power to produce it'. The 'will' on this type of account sits with a kind of causal role between the desire and the act aimed at fulfilling the desire. An example of a 20th century attack on this concept of the 'will' is that of Wittgenstein (1958), especially in his *Notebooks* and his *Philosophical Investigations*. He argued that an act of will is not the cause of the action, but is the action itself, and that voluntary action need not involve an 'act of volition' - it may involve 'no more than behaving in a familiar sort of way' – 'I raise my arm without feeling surprised' (Philosophical Investigations §611–§628). Hyman, in his review, concludes that in Wittgenstein's attempt to extricate the 'will' from the traps that can be laid by the way we use language, the act of 'willing' as such vanished altogether.

According to the Oxford English Dictionary a 'preference' is a greater liking for one alternative over another'. The use of separate words in the CRPD suggests there is a distinction to be made between the 'will' and a 'preference'. A problem that may arise especially in the case of mental health disabilities is a divergence between a person's 'will' and a current 'preference'.

A distinction between 'will' and 'preference', I suggest, is consistent with significant recent accounts of 'the will' that resemble in form an aspect of Kant's concept of the 'will', summarised by Schneewind (2004) as follows:

> The will, then, as distinct from the ability to choose, is the capacity to transform felt urges or desires with causal force into motivating reasons for action with justifying validity. To possess a will is therefore also to be able to test desires to see whether or not they can be validated as reasons'.

Kant's idea of the 'will' forms part of a larger account including the choice of 'ends', but this is not relevant for our purposes. The idea shared with a number of recent accounts is of the 'will' as a kind of higher-order motivating structure that determines which desires become intentions.

Examples of recent accounts of the 'will' are those of Gary Watson (2003), Michael Bratman (2000) and Pamela Hieronymi (2009). Watson has noted that:

> 'Deciding to' typically involves shaping priorities among a structure of reasons and thereby giving certain considerations a special reason-giving force. And this is precisely what is required for a substantive executive role: that the executive have latitude for its own operation within a legislative framework to which it is subordinate. On the other hand, the will would not be carrying out practical reason if it went against its mandates. When intention fails to be guided by judgment, it fails to operate in its executive capacity—it fails to operate as a will.

Bratman's account of the 'will' sees it again as a higher-order self-governing mechanism, one in which 'values' play a key role. Desires are subject to forms of deliberation within higher order 'policies' that extend over time and that express commitments towards ends that embody values. Within this self-governing framework, desires justifying action have a 'reason-providing' quality. Hieronymi gives an account of 'will' as 'reason in its practical employment':

> I believe the claim that willing to ϕ [where ϕ is an action] is or centrally involves settling the question of whether to ϕ, where that settling is the kind of activity done for reasons, amounts to a liberal interpretation of the claim that willing is an exercise of our capacity for reasoning — our capacity to raise, deliberate about, and answer questions. … A person's answer to a question, and his or her reasons for it, will cohere, to a greater or lesser extent, with the person's other beliefs, judgments, intentions, and general temperament.

From these accounts a distinction can be drawn between the 'will' - as a higher-order function reflective about values and what it would be best to do, as opposed to desires or inclinations - or 'preferences'. 'Will' and 'preferences' normally, by and large, run together. It is when they diverge and a person needs to make a serious decision that a problem may arise. The 'will', on this view, can be seen as founded on a person's deeply held, reasonably stable and reasonably coherent personal values. In this sense, it is not the same as a desire, inclination, or a currently held 'preference', even a strongly expressed one.

4.1. A case example

> Anne is 23 years old and a talented student at a prestigious school of drama. She has a diagnosis of a bipolar illness. Her mother committed suicide during a depressive episode when Anne was 14, and she has other relatives with the same condition. She lives with her father, a retired dentist. Her married sister, Louise, lives nearby. Anne has a close relationship with both, as well as Louise's husband.
>
> Anne's education, especially during her final year at school, has been badly affected by her illness, and she is determined to finish her drama course. The drama school knows about Anne's condition and has been very supportive thus far. Following recovery after a second involuntary admission to a psychiatric hospital Anne composed an advance statement based on her previous experiences of manic episodes: 'If I am unable to sleep for more than three hours on two consecutive nights; or, if I go wandering in the centre of town after midnight; or, if I say I am a resurrection of Sarah Bernhardt, then I need to be treated - against my will, if that becomes necessary. If I refuse, I accept that I should be admitted to hospital on a compulsory order so I can be treated promptly in order to minimize the length of my relapse.'
>
> Six months later, unable to sleep and flimsily dressed, Anne went out wandering around 2 a.m. She was picked up by the police four hours later. She had been loudly reciting long passages from "Who's Afraid of Virginia Woolf' to all and sundry. She claimed she was a great actress.
>
> The police, using a power under the Mental Health Act that allows a person in a public place who appears to have a mental disorder to be taken to 'place of safety', escorted Anne to the Accident and Emergency Department of the local hospital. There her records revealed her 'advance statement'. Her father was contacted and soon arrived at the hospital.
>
> Anne recognized the 'advance statement' as having been formulated by her, but said she had now changed her mind. Drama school was no longer important, she said. She now realised she was 'hugely

> gifted and inspired, miles ahead of the others'. She was sure she would soon be offered major roles on the stage or cinema. The only serious dilemma for her will be deciding which to choose. Being in the city in the early hours, she said, could be frightening, but the rewards were a heightening of her creative powers and a 'fantastic preparation for the great parts coming my way … being able to enthrall strangers in the street with my powerful declamations shows how great an actor I am'. When asked if she was Sarah Bernhardt she said: 'I'm not certain … but probably'.
>
> Anne refused any treatment intervention and said she did not require any help with making any decisions and did not want to discuss the matter further.

Especially difficult in determining a person's 'will and preferences' are those where there has been a sudden, significant change in the person's outlook, especially where an expressed 'preference' clashes with the previously expressed 'will' – where they become 'out of joint'. Anne's situation is an example. Which "preferences' should we respect? Should it be those stated in her advance statement or those that she is now expressing? Is one more worthy of respect or 'authentic' than the other? If so, which one?

I suggest that Anne's 'will and preferences' expressed in her advance statement is the more 'authentic'. The 'will' expressed represents a deep commitment to completing her course and becoming an actress. (Many past statements and actions by Anne, not mentioned in her advance statement were consistent with this commitment). The advance statement was made at a time when Anne was able to calmly reflect on what was especially important for her. The 'preference' expressed for early treatment so as not to jeopardize her training is entirely consistent with her 'will'. Thus it would be justified to over-ride her presently expressed 'preference' – one based on a likely baseless belief that she is already 'great'.

If this analysis is accepted, it would follow that we would act similarly if Anne had not made a written advance statement, but had expressed through various comments and life choices, the same values (or 'will') and associated 'preferences', as evidenced, for example, by her father, sister and brother-in-law. Even if Anne had not previously verbalized a clear treatment preference, her previously expressed 'will' ascertained from her deep commitments and life choices would have to count heavily in deciding whether or not to respect her present 'preference'.

Anne's resistance to the idea of support with making a decision did not allow any discussion of her 'preference' and its relationship to her 'will'.

If will and preferences can diverge in this manner, what then are the implications for the meaning of 'substitute decision-making'? We need to go back to the fundamental aims of the Convention. The first 'General Principle' refers to:

> Respect for inherent dignity, individual autonomy including the freedom to make one's own choices, and independence of persons

The statement seems to point to a primary focus on the person's 'will', as I have characterized it, as arguably it has the strongest relationship to personal autonomy and the freedom to make choices. Autonomy requires an ability to value.

5. Some implications for practice

There are four implications that merit comment, though the dictates of space allow only a brief account at this stage. 1. How are we to determine (or 'interpret') a person's 'will' and 'preferences'? 2. How can we deal with the fact that sometimes people may change their 'will' profoundly? 3. Are there situations where we might decide not to privilege a person's 'will' against current 'preferences' even though the latter appear to be inconsistent with that 'will'? 4. What should we do in situations where we cannot determine a person's 'will', for instance, because they may not have had the ability to ever express it, or because circumstances preclude our knowing it?

5.1. 'Interpreting' a person's 'will' and 'preferences'

This involves a form of 'folk' or 'common-sense' psychology which has been important in the philosophy of mind. Some philosophers call it 'interpretation' (see for example, Glover (2014)). It describes, explains and predicts behaviour in everyday terms of intentional states such as beliefs and desires. We are thus able to understand other minds. Dennett (1991) characterized this ability as follows: '… for all of its blemishes, warts and perplexities, folk psychology is an extraordinarily powerful source of prediction. It is not just prodigiously powerful but remarkably easy for human beings to use. We are virtuoso exploiters of not so much a theory as a craft.'

An influential account of this ability is found in Donald Davidson's method of 'Radical Interpretation' (Davidson, 1973, 1994) whose application in the context of 'decision-making capacity' has been described by Banner and myself (Banner & Szmukler, 2014). 'Interpretation' along these lines is valuable, it is argued, in assessing whether an apparently imprudent or bizarre treatment decision is a result of an impairment of decision-making capacity or not (or, following our discussion above, a disjunction between a person's 'will' and 'preferences'). Davidson's 'Principle of Charity' starts from the position of adopting a presumption that what a 'radically unfamiliar' speaker says and does is by and large 'coherent' with their beliefs and values, and that their beliefs generally 'correspond' to facts about the real world. When that 'coherence' or 'correspondence' breaks down – when a contemporaneous belief or value, for example, is not understandable in the holistic context of the person's larger set of related beliefs and values – we begin to suspect an undermining of decision-making capacity (or of the relationship between 'will' and 'preferences'). Kim's argument that the 'ability to value' may be an important element of decision-making capacity is based on similar considerations (Kim, 2016), while a detailed analysis of 'interpretation' in relation to 'disordered minds' is given by Glover (2014). There are obvious relationships here to Karl Jasper's notion of 'understandability' (Jaspers, 1963), and, as Banner (2010) points out, with Wittgenstein's analysis of 'rule-following' as a broad responsiveness to reasons that is embedded in social practice, in the shared life we inhabit as intentional agents. We cannot help but see intention and hear meaning when observing the behaviour of, or listening to, another person. However, Wittgenstein argued, this practice is not amenable to a set of objective, codified principles or standards of rationality. Nevertheless, we know whether a word, for example, is being used in the 'right' way.

Certain normative commitments are entailed by having a belief: for example, not simultaneously asserting its opposite, or acting in a way that flatly contradicts one's holding that belief. A belief or value also derives its meaning from its relations to other beliefs and values. These accounts of 'interpretation' argue for an underlying normativity in our mental functioning. Without such a normativity, embedded in shared practice, and one which cuts across cultures - despite a diversity of values - our natural ability to understand others could not exist.

'Interpretation', especially in difficult cases, may require support for the person - sometimes very substantial - as well as a number of safeguards. For example, the decision must involve people who know the subject well; an independent advocate should be available; where there is a dispute, referral to a court or tribunal must be available.

The UN CRPD Committee in its General Comment on Article 12 in fact used the word 'interpretation' when stating that where, after significant efforts have been made, it is not practicable to determine the will and preference of an individual, a 'best interpretation of will and preference' should be made. The analysis proposed above addresses situations where there is a difficulty in such a determination.

A recent decision[6] in the English Court of Protection, in which a woman made the unusual treatment decision to reject renal dialysis even though the prognosis for recovery was good, is an excellent example of respect being accorded to a person's highly unusual though deeply held personal values - or 'will' (Ryan, Szmukler, & Large, 2016).

The analysis also leads directly to an alternative formulation of 'decision-making capacity' and 'best interests'. Both terms have been contested, especially the latter, which has been conceptualized in a number of ways (McCubbin & Weisstub, 1998; CRPD Committee, General Comment 1). One can now think about impaired 'decision-making capacity' as being marked by a significant inconsistency between a person's 'will' – assessed holistically - and a contemporaneous 'preference'. Anne's 'preference' for no intervention set against her advance statement in the case cited is an example. 'Best interests' can be understood as giving effect to the person's 'will'. An intervention in the person's 'best interests' aims at facilitating the expression of the person's 'will', his or her deep values and commitments (Szmukler & Bach, 2015). It can be argued that on this interpretation giving effect to a person's 'will' - even against a present 'preference' – does not constitute, in an important sense, a 'substitute decision'.

The approach proposed might be termed as reflecting a 'decision-making capability-will and preferences' framework.

The decision of the German Federal Constitutional Court, discussed in Section 2.1 above, with its distinction between 'free will' and 'natural will' shares some similarities with the position proposed here. The attempt in this paper is to analyse in substantial depth the difference between these forms of the 'will' – or between a 'will' and a 'preference' – and the role of the method of 'interpretation' in their assessment.

5.2. Changes in beliefs and values (the 'will')

A potentially problematic aspect of the description of 'will' above concerns how we might recognize changes in a person's deep commitments and values that do not seem problematic. In the main, changes in values are gradual, by and large 'coherent' - in relation to the person's associated beliefs and values - and 'understandable' when viewed in the context of the person's life circumstances. However, there appear to be fairly rare instances where a sudden, 'quantum' change in persons' values can occur (C'de Baca & Wilbourne, 2004; Miller, 2004). These have been little studied so what we know about them is limited. In perhaps the major study to date, such a change was noted to be triggered by a sudden, apparently unanticipated mystical or revelatory insight usually with a spiritual character. The most commonly reported changes were: relief from fear, depression, anger; release from destructive behavioral patterns; a deepening or healing of relationships; spirituality becoming vibrant and trusting and central to their lives; self-actualization; affirmation of a sense of self; and a trust in the future. While it is not clear how deep the value changes went, the impression one gains is of a new but nonetheless generally 'coherent' set of beliefs and values resembling those held by people with deep religious beliefs. Changes were generally maintained at a 10 year follow-up though 'the QC [quantum change] experiences fell along a continuum, with some describing their QC as a one-time event that dramatically changed the way they lived, others experienced repeated changes that were either of similar intensity to the initial experience or milder than the initial experience, and some framed the initial change experience as one aspect of a continuing growth process' (C'de Baca & Wilbourne, 2004).

A second type of change in the 'will' may occur in people with a diagnosis of a mental illness such as schizophrenia whose beliefs and values may change substantially over time from those previously held, but which assume a level of stability, 'coherence' and 'correspondence' that leads one to conclude that these now constitute the person's real 'will'. An excellent account of how we might determine whether such a state has been reached has been given by Glover (2014). Another recent case heard in the Court of Protection respected the long-standing unusual religious beliefs - despite their grave consequences - of a man with a long history of schizophrenia, with the judge making the memorable statement: 'It is no more meaningful to think of Mr B without his illnesses and idiosyncratic beliefs than it is to speak of an unmusical Mozart'.[7]

5.3. When the 'will' might not be privileged

In instances like Anne's, where the disjunction between 'will' and 'preference' is likely to be temporary, a good case can be made to support the person's previously expressed 'will'. The disjunction is temporary because the underlying disturbance of mental functioning is reversible with treatment, or, sometimes in time, without. Where the person has an irreversible condition, like a dementia, the objection can be made that the person's previously expressed 'will' cannot be regarded as the 'will' of the now 'different' person with dementia. In such cases, one might decide to respect the person's 'preference' even though it is inconsistent with the person's previous 'will'. An example might be the possibility of respecting the 'preference' of a life-long, committed vegetarian, now severely demented, to eat meat, especially if he becomes highly agitated if it is refused. What should be respected here will usually demand a case by case decision, often very difficult, based on a discussion involving those who know the person well.

Just as 'will' and 'preferences' may diverge, so may 'will' and rights. When a 'right' might be judged to override a clearly formulated 'will' poses a more familiar question (perhaps of 'safeguarding' or 'protection' versus 'empowerment' or 'autonomy'), though a highly controversial one. Only a few points, relevant to the main theme of the paper, will be made here.

An example of the problem is whether a right to enjoy freedom from exploitation should override a person's 'will' to live alone in a situation where such a right is threatened. The approach outlined in this paper suggests that the quandary should be resolved by achieving a 'best interpretation' of whether the person's 'will' to live independently - as judged on the basis of his or her beliefs, values and important life goals – would be consistent with accepting the level of hardship or risk to which the person would be exposed (after appropriate support services were provided). A recent case heard in the English Court of Protection turned largely on this type of consideration. The court supported the return to her home - at least for a substantial trial period - as opposed to being kept against her wishes in a care home, of a strong-minded woman with dementia who would run considerable risks to her physical safety at home. This decision was largely founded on the strength of her previous values and life choices (consistent with her current preference) and an advance statement in which she prioritized her quality of life over its prolongation.[8] In the words of the judge: 'On a personal level, her strong sense of self, her belief in the importance of the individual, her desire for freedom and autonomy are magnetic factors, operating at positive and negative poles by providing both the pull of freedom and the counterforce of resistance to outside care.'

An especially thorny predicament may arise when a person refuses a treatment that may be life-saving. Such a refusal may be expressed, for example, in an advance directive meeting all of the criteria for its validity, including the accurate anticipation of the circumstances now in play. A 'right to life' may then be set against a right to have one's clearly formulated 'will' respected. Fortunately such cases are rare (Swanson, Swartz, Elbogen, et al., 2006). In the end, states will legislate for such cases according to which values are prized most. What is crucial though, from the standpoint of the CRPD, is that persons with disabilities are treated 'on an equal basis' with those without; for example, the 'will' of a person with a mental disorder is to be respected in the same way as the 'will' of a person with a physical disorder.

[6] King's College Hospital NHS Foundation Trust and C. [2015] EWCOP 80.

[7] Wye Valley NHS Trust v B. [2015] EWCOP 60.

[8] Westminster City Council v Manuela Sykes. [2014] EWHC B9 (COP).

5.4. When one cannot be sure about the 'will'

There are situations where it may be impossible to know what a person's 'will' might be in relation to an intervention. Examples include a person who is unconscious or is in an organic confusional state and there is no-one available who knows the person and who is thus able to provide some guidance about what the person's attitude might be; or a person with a severe intellectual disability who may not have been able to clearly express a coherent 'will' (though there may be fragments of observed behaviour and utterances pointing to what has been important to the person that offer some indication).

The Australian Law Reform Commission (2014) published a report which examined 'Equality, Capacity and Disability in Commonwealth Laws' that offered an interesting view about the relationship between 'rights' versus 'will and preferences':

> The inclusion of 'rights' is the crucial safeguard. In cases where it is not possible to determine the will and preferences of the person, the default position must be to consider the human rights relevant to the situation as the guide for the decision to be made. (3.53)

An example of such a case might be a person found wandering aimlessly through traffic who is in an 'organic confusional state', is unknown to anyone in the vicinity, where there are no clues as to anyone who could be contacted for any information about the person, and who resists any intervention.

6. Meaning of 'respect'

A point needs to be made about the meaning of 'respect' in the phrase that one 'must *respect* the rights, will and preferences of persons with disabilities'. This was the subject of much discussion at a recent workshop in London organised jointly by the Essex Autonomy Project and the Ministry of Justice on whether the Mental Capacity Act 2005 was compliant with the CRPD (Martin, Michalowski, Jutten, & Burch, 2015). It was concluded that 'respect' for the rights, will and preferences cannot mean that one must *accede to*, or *agree to abide by,* the person's 'rights, will and preferences'. This must be so because those three elements – 'rights', 'will', and 'preferences' – may point in different directions.

7. Conclusions

In thinking about whether any intervention should be made in the lives of people with disabilities, the CRPD requires that we 'respect the rights, will and preferences' of the person. Some authorities, including the CRPD Committee, have concluded that this must be taken to mean that involuntary treatment is effectively ruled out. What is missing to date from the argument for such an injunction is an analysis of the meaning of the words 'respect', 'will', and 'preference'. I have attempted to provide such an analysis based on philosophical discussions of the 'will' and the place of 'folk psychology' (or 'interpretation'). If a distinction can be drawn between 'will' and 'preference', and they point in different directions, then a problem arises concerning which should be 'respected'. The 'will', taken as representing a person's deep beliefs, values and commitments should in general be respected (though there might be exceptions, for example, where a person has an irreversible mental impairment). Determining what constitutes a person's 'will and preferences' involves the method of 'interpretation'. Those who regard this method as lacking 'objectivity' might find P Strawson's remarks on the subject fairly salutary:

> Still, we have the theoretical idea of the two histories, each complete in its own terms; we might call them the physical history and the personal history… Each story will invoke its own explanatory connections, the one in terms of neurophysiological and anatomical laws, the other in terms of what is sometimes called, with apparently pejorative intent, "folk psychology", i.e. the ordinary explanatory terms employed by diarists, novelists, biographers, historians, journalists and gossips, when they deliver their accounts of human behaviour and human experience – the terms employed by such simple folk as Shakespeare, Tolstoy, Proust and Henry James. (Quoted in Glover, 2014, p87)

Difficult situations may also arise when a person's carefully and clearly expressed 'will' is in conflict a 'right' or 'rights'.

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